



OET Mini Listening Test #1

This test contains one OET Listening Part A extract, six Part B extracts and one Part C extract.

Please note that this mini test is for practice purposes and is not a complete OET Listening Test.

Find complete sample tests at [oet.com](https://www.oet.com)

Contents

Cover page.....	1
Part A	2
Part B	3
Part C	5
Audio script	7
Simple answer key	14
Extended answer key	15

Audio file

Access the Mini Test audio here:

 [OET Mini Listening Test #1 Audio](#)

LISTENING: Mini-Test 1

This test contains one Part A extract, six Part B extracts and one Part C extract.

Please note that this mini test is for practice purposes and is not a complete OET Listening Test.

CANDIDATE NAME: _____

D.O.B.:

D	D	M	M	Y	Y	Y	Y
---	---	---	---	---	---	---	---

PROFESSION: _____

VENUE: _____

TEST DATE: _____

Starting at the left, print your Candidate Number and fill in the corresponding circle below each number using a 2B pencil.

Example:

2	5
0	0
1	1
2	2
3	3
4	4
5	5
6	6
7	7
8	8
9	9

CANDIDATE NUMBER									
0	0	0	0	0	0	0	0	0	0
1	1	1	1	1	1	1	1	1	1
2	2	2	2	2	2	2	2	2	2
3	3	3	3	3	3	3	3	3	3
4	4	4	4	4	4	4	4	4	4
5	5	5	5	5	5	5	5	5	5
6	6	6	6	6	6	6	6	6	6
7	7	7	7	7	7	7	7	7	7
8	8	8	8	8	8	8	8	8	8
9	9	9	9	9	9	9	9	9	9

CANDIDATE DECLARATION

By signing this, you agree not to disclose or use in any way (other than to take the test) or assist any other person to disclose or use any OET test or sub-test content. If you cheat or assist in any cheating, use any unfair practice, break any of the rules or regulations, or ignore any advice or information, you may be disqualified and your results may not be issued at the sole discretion of CBLA. CBLA also reserves its right to take further disciplinary action against you and to pursue any other remedies permitted by law. If a candidate is suspected of and investigated for malpractice, their personal details and details of the investigation may be passed to a third party where required.

CANDIDATE SIGNATURE: _____

TIME: APPROXIMATELY 40 MINUTES

INSTRUCTIONS TO CANDIDATES

DO NOT open this question paper until you are told to do so.

One mark will be granted for each correct answer.

Answer **ALL** questions. Marks are **NOT** deducted for incorrect answers.

At the end of the test, you will have two minutes to check your answers.

At the end of the test, hand in this **Question and Answer Booklet**.

You must not remove OET material from the test room.

HOW TO ANSWER THE QUESTIONS

Part A: Write your answers on this **Question and Answer Booklet** by filling in the blanks. Example: Patient: Ray Sands

Part B & Part C: Mark your answers on this **Question and Answer Booklet** by filling in the circle using a 2B pencil. Example: (A)

(B)
(C)

Extract 1: Questions 1-12

You hear a rheumatologist talking to a patient called Emil Egan. For **questions 1-12**, complete the notes with a word or short phrase that you hear.

You now have thirty seconds to look at the notes.

Patient Emil Egan

Onset of condition (2 years ago)

- pain in back
 - attributed to work as a (1) _____
 - pain in lower spine described as 'niggling'
- did (2) _____ and weightlifting at gym to strengthen back
- pain gradually worsened - mostly located around the (3) _____ (left)
- accompanied by general feeling of (4) _____ in the back
- sometimes radiates to left (5) _____ and leg
- pain in left (6) _____ on one occasion

Treatment

- OTC painkillers ineffective
- initial diagnosis of (7) _____
- prescribed anti-inflammatory medication
- MRI scan unremarkable

Referral to osteopath

- (8) _____ – provided relief
- has commenced regular massage – no immediate effect
- has been recommended (9) _____ – not commenced

Current situation

- investigations to rule out (10) _____ (sister has recent diagnosis)
- diagnosed with (11) _____, confirmed by X-ray
- n.b. Mother died of cancer of (12) _____

That is the end of Part A. Now look at Part B.

Part B

In this part of the test, you'll hear six different extracts. In each extract, you'll hear people talking in a different healthcare setting.

For **questions 13-18**, choose the answer (**A**, **B** or **C**) which fits best according to what you hear. You'll have time to read each question before you listen. Complete your answers as you listen.

Now look at question 18.

Fill the circle in completely. Example: ☒ C

13. You hear a community pharmacist talking to a patient.

The patient agrees to

- ☐ A change the type of medication he takes.
- ☐ B avoid taking his medication with meals.
- ☐ C take his medication at a different time.

14. You hear a hospital nurse talking to a pharmacist.

What is the pharmacist doing?

- ☐ A suggesting various solutions to a problem
- ☐ B trying to identify why an incident has occurred
- ☐ C establishing that correct procedures have been followed

15. You hear a hospital nurse talking to a patient who is being discharged following hip-replacement surgery.

How does the patient feel about the modifications made to his home?

- ☐ A unconvinced that they're actually necessary
- ☐ B surprised they aren't intended for long-term use
- ☐ C concerned that he hasn't been shown how to use them

16. You hear a hospital doctor talking to the grandson of one of her patients.

The man is asking for information about

- (A) how likely his grandfather is to make a full recovery.
- (B) why his grandfather's condition has changed so rapidly.
- (C) what support will be available once his grandfather is discharged.

17. You hear an emergency doctor briefing his team about a patient who is about to arrive in his department.

What is he doing?

- (A) assigning roles to members of the team
- (B) outlining the initial procedure they should follow
- (C) explaining what information they need to obtain from paramedics

18. You hear a hospital doctor talking to a nurse about a patient.

What do they agree to do for the patient today?

- (A) refer him to a consultant for a senior review
- (B) initiate some therapy to help with his breathing
- (C) monitor his progress on the existing treatment plan

That is the end of Part B. Now look at Part C.

Part C

In this part of the test, you'll hear an extract. In this extract, you'll hear a health professional talking about aspects of their work.

For **questions 19-24**, choose the answer (**A**, **B** or **C**) which fits best according to what you hear. Complete your answers as you listen.

Now look at extract one.

(A)
(B)
Fill the circle in completely. Example: (C)

Extract 1: Questions 19-24

You hear a presentation by Dr Nihal Premadasa, who is talking about a weight-management programme for patients with acquired brain injury.

You now have 90 seconds to read **questions 19-24**.

- 19.** What aspect of recovery from a brain injury does Nihal feel gets too little attention?
- (A) the extent to which the patient's self-image is affected.
 - (B) the way it affects the patient's relationships with others.
 - (C) the effect on the patient's ability to perform everyday tasks.
- 20.** What led Nihal's team to focus on the issue of weight change with their patients?
- (A) the impact it has on the effectiveness of other therapies
 - (B) the risk that it can pose to patients' long-term health
 - (C) the number of patients expressing concerns about it
- 21.** The programme set up by the team differed from those delivered elsewhere in terms of
- (A) the level of training available to team members.
 - (B) the emphasis placed on decision making by patients.
 - (C) the wide range of resources it was able to make use of.

22. What aspect of the programme was changed after its launch day?

- ☐ (A) how patients were recruited to it
- ☐ (B) how patients kept a record of their progress
- ☐ (C) how long patients were given to engage with individual activities.

23. In sessions focussing on the loss of taste and smell, the team encouraged patients to

- ☐ (A) consider various other attributes of food.
- ☐ (B) develop a tolerance of a wider range of foods.
- ☐ (C) become aware of the extent of their impairment.

24. What positive outcome of the programme was highlighted in weekly patient feedback?

- ☐ (A) the satisfaction of meeting measurable weight loss targets
- ☐ (B) the benefits gained from working with others in the sessions
- ☐ (C) an increased willingness to engage in a range of therapeutic activities

End of test.

Mini Listening Test 1 – Audio Script

OCCUPATIONAL ENGLISH TEST. MINI LISTENING TEST 1

This test has three parts. In each part you'll hear a number of different extracts. At the start of each extract, you'll hear this sound: ---***---.

You'll have time to read the questions before you hear each extract and you'll hear each extract **ONCE ONLY**. Complete your answers as you listen.

Part A

Extract one. Questions 1 to 12.

You hear a rheumatologist talking to a patient called Emil Egan. For questions 1 to 12, complete the notes with a word or short phrase that you hear. You now have thirty seconds to look at the notes.

PAUSE: 30 SECONDS

---***---

Rheumatologist: So, Emil I understand you've been having problems with your back. I've got your notes here, but it would be good to hear your perspective. Could you run through how this started, what treatment you've had and anything else you think I should be aware of.

Emil Egan: Sure. I've been getting back pain for around two years. It all started when I got a job as a bus driver. I've always done a lot of driving, but it had never been my main occupation before. Anyway, after about four months, I realised I was starting to get a few niggling pains in my lower spine – nothing too serious, but enough to make me feel a bit uncomfortable at the end of a day's work. So, I started going to the gym – because I've always been pretty fit. I'd do stretching sessions along with lots of weightlifting because I wanted to strengthen my back. I mean, it seemed like a good idea at the time.

Rheumatologist: Sure.

Emil Egan: Anyway, things continued to get gradually worse until it got to the point where as soon as I sat down anywhere, I'd feel the pain come on. It's mainly just above the glute and always on that left side. I don't know how best to describe it really, but my back just doesn't feel right – I guess you'd say there's some kind of stiffness there. Anyway, the thing about this pain is that it tends to move about a bit. It's always on that side, but sometimes it spreads up and down, so that I feel it in my buttock and once or twice I've woken up with the pain going all the way down my leg – and on one occasion as far as the heel. Other times I get some pain higher up in my back too. In the end, it got so bad that I had to give up the job – that was about six weeks ago.

Rheumatologist: I see. And what about treatment?

Emil Egan: Well, I tried all sorts of over-the-counter painkillers and none of them touched it – at first my doctor said that it was probably sciatica, and that it would probably go away of its own accord. I've also been using heat packs and ice, but they only make me feel better – they don't solve anything. Anyway, eventually the

doctor put me on an anti-inflammatory drug and referred me for some tests. I had an MRI scan, but that didn't show up anything significant.

Rheumatologist: I see. And have you had any therapy?

Emil Egan: Yes, I was referred to an osteopath. As well as manipulating my back a bit, she suggested various treatments. I started shockwave therapy, and that made an immediate difference. I felt so much better that I was able to start doing simple strengthening exercises again, and walking got a lot easier. Another thing she suggested was massage, and I've been getting that regularly too. I'm not sure how much difference it makes though. The other thing she suggested was acupuncture. I'm not sure about that – so I haven't followed it up. She also got me to try different pillows – you know a hard one or two soft ones, but I didn't notice much difference, and I've now bought myself an adjustable bed, that's certainly more comfortable and I'm sleeping better – but the pain hasn't gone away.

Rheumatologist: And you've now had a diagnosis I believe.

Emil Egan: Yes, that's right. My sister has developed fibromyalgia and I was concerned I might have it too. So, I talked to my doctor who said that it was unlikely given my symptoms, but he did send me for a blood test and an X-ray. Anyway, to cut a long story short, at that point, I was diagnosed with osteoarthritis. I wasn't very happy about that, but apparently, the X-Ray showed that I've lost quite a bit of cartilage. And that brings us more or less up-to-date. The other thing that I ought to mention is that my mother died of cancer earlier this year. It was in her jaw, and that's pretty awful to deal with, so it's been a difficult time for me generally.

Rheumatologist: Yes, I can appreciate that. OK Emil, thank you for all that background. What I'd like to do next ... [fade]

PAUSE: 10 SECONDS

That is the end of Part A. Now look at Part B.

Part B

Part B

In this part of the test, you'll hear six different extracts. In each extract, you'll hear people talking in a different healthcare setting.

For questions 13 to 18, choose the answer A, B, or C which fits best according to what you hear. You'll have time to read each question before you listen. Complete your answers as you listen.

Now look at Question 13. You hear a community pharmacist talking to a patient. Now read the question.

PAUSE: 15 SECONDS

___***___

Female: Hi – what can I do for you?

Male: I was after something for this pain I'm getting in my chest. It comes on after dinner, and I get like an acidic taste at the back of my throat too. It's been going on about a week. I mean, I sometimes eat late-night takeaways, you know, curries and stuff – and that can give me indigestion – but never anything like this.

Female: I see. Are you taking any medication?

Male: Yeah, I've just started taking doxycycline for my acne – I have that with my evening meal.

Female: OK – what you're describing sounds like heartburn. Doxycycline can sometimes cause that if you lie down right after taking it, so I'd recommend having it with breakfast instead. Spicy foods can also worsen heartburn, so I'd keep off those for the time being.

Male: Sure. Now you mention it, the doctor did say to avoid lying down after taking my medicine – I'd forgotten that.

PAUSE: 5 SECONDS

Question 14. You hear a hospital nurse talking to a pharmacist. Now read the question.

PAUSE: 15 SECONDS

---***---

Male: Hi, I wanted to talk to you about a medication error.

Female: Sure. What happened exactly?

Male: Well, I was administering medication to a patient, and I accidentally gave them the wrong dose.

Female: I see. What medication was it?

Male: It was insulin. The dose should've been one unit, but I accidentally gave ten units.

Female: Okay, thanks for letting me know. Did you report the incident?

Male: Yes, I reported it to the lead nurse right away.

Female: Good. We'll need to fill out an incident report as well. Can you tell me what steps you took after you realised the error?

Male: Yes, I monitored the patient's blood sugar levels closely and notified the physician right away. The patient didn't experience any adverse effects, thankfully.

Female: That's fortunate. In the future, if you're ever unsure about a medication order, don't hesitate to clarify it with the prescriber.

Male: I will. But it won't happen again – I've learned my lesson.

PAUSE: 5 SECONDS

Question 15. You hear a hospital nurse talking to a patient who is being discharged following hip-replacement surgery. Now read the question.

PAUSE: 15 SECONDS

___***___

Female: So, let's just go over what you've discussed with the different therapists today – just to be sure we've covered everything and you're completely happy with the arrangements.

Male: Right, well, the physiotherapist's given me some exercises and I know that's not something I can afford to skip. Then the occupational therapist has arranged for some grab rails and things like that to be fitted at home – and I think somebody's already been round to see to that.

Female: They have.

Male: Anyway, my son's been shown where they are and what they're for – so that shouldn't be a problem. But then she also said it was actually up to me how long I use them for – that I need to see how I get on. I was a bit taken aback by that, to be honest. But I do get the point that, ideally, I need to learn to manage without them – so I'll do my best.

Female: Great.

PAUSE: 5 SECONDS

Question 16. You hear a hospital doctor talking to the grandson of one of her patients. Now read the question.

PAUSE: 15 SECONDS

___***___

Female: Hello, I'm the doctor who's looking after your grandfather – I think you asked to speak with me?

Male: Yes, thanks. It's just that I'm really worried about him because I only saw him last week and he was fine. I got a call to say he was in the hospital with an infection and he was a bit disoriented, but I was away on a business trip – so it's taken a couple of days to get over here.

Female: Sure.

Male: What's really worrying me is that he doesn't even recognise me. I mean, he was fine last week – and there's been this sudden deterioration. You see, he's the main carer for my grandmother and we've had to bring professional carers in to look after her. That's OK – but I need to get some idea of where this is heading – do I need to make permanent arrangements for her – or is there any chance that he might get over this?

Female: OK – I understand your concerns – let's sit down and talk this through... [fade].

PAUSE: 5 SECONDS

Question 17. You hear an emergency doctor briefing his team about a patient who is about to arrive in his department. Now read the question.

PAUSE: 15 SECONDS

---***---

Male: Okay, so what's going to happen is, we've been put on pre-alert by the ambulance control room because we've got a 34-year-old male patient coming in. Apparently, he's fallen three metres from a ladder – his workmate reports that he was blow-torching a felt roof and overstretched – so it seems to be a mechanical injury. As well as head injuries, he has a laceration to his right leg and a burn on his left arm. Subsequently, he's had a seizure in the ambulance of around two minutes' duration.

So, we're going to keep him on the ambulance stretcher while we go through the standard IMIST handover, then we're going to move him onto our stretcher. At that point, you can each begin your individual assessments that you'll feed back to me. While you're doing that, I'll get a thorough handover from the ambulance crew so that we've got full details before we decide what we're going to do.

PAUSE: 5 SECONDS

Question 18. You hear a hospital doctor talking to a nurse about a patient. Now read the question.

PAUSE: 15 SECONDS

---***---

Female: So, Mr. Clews is a 45-year-old male with a history of diabetes and hypertension who was admitted with pneumonia. How's he doing today?

Male: Unfortunately, he's still presenting with a persistent cough and shortness of breath, and his oxygen saturation levels aren't improving as we'd hoped.

Female: I see.

Male: We've been treating him with antibiotics and nebuliser treatments, and the respiratory therapist's also been working on his breathing.

Female: Have we checked his blood glucose and blood pressure levels recently?

Male: His glucose is within the normal range, but his blood pressure's a bit high.

Female: I see. I'll adjust his hypertension medication accordingly, but we may also need to consider other treatment options, like corticosteroids or oxygen therapy. Have you talked to the pulmonologist?

Male: I have. She suggested sticking to the current plan for another day or so, and keeping a close eye on him. If there's no improvement, we may then need to consider additional interventions.

Female: Sure. Thanks.

PAUSE: 10 SECONDS

That is the end of Part B. Now look at Part C.

Part C

Part C

Questions 19 to 24. You hear a presentation by Dr. Nihal Premadasa, who is talking about a weight-management program for patients with acquired brain injury.

You now have 90 seconds to read questions 19 to 24.

PAUSE: 90 SECONDS

---***---

Dr Nihal Premadasa: Hi. My name's Nihal Premadasa. I lead a multidisciplinary team at a residential rehabilitation center for adult patients with acquired brain injury. Today I'd like to tell you about a twelve-week weight-management program we developed for our patients.

Firstly, some background about our center and the program itself. A brain injury can be devastating for the patient, who often experiences a loss of identity. The impact of that can be worsened by the fact that there's a general lack of public understanding about the nature and effects of brain injuries. Patients often face a long struggle to regain physical, cognitive, and social skills, and that's where a residential center like ours comes in. Our multidisciplinary team aims to enhance patient recovery, and we identified that an often-overlooked area of that recovery was how a person feels about themselves as they adapt to a new lifestyle with the physical, emotional, and cognitive scarring caused by their injury – and weight change can play a part in that.

Weight change is common during recovery from a traumatic brain injury for several reasons, including the use of medication and long-term cognitive or behavioral difficulties. We realised, however, that disability is the main contributing factor to increased weight gain amongst our residents, and this, in turn, often leads to a diagnosis of type-2 diabetes. For this reason, we decided to launch a weight-management program to improve the general well-being and health outcomes of our patients and to address that issue in particular. We did this by concentrating on short-term goals – supporting their efforts to achieve a realistic target weight and increasing their level of physical activity generally.

So how did we go about setting up the program? Firstly, we obtained funding to attend a train-the-trainer program in weight management. This enabled us to develop a twelve-week lifestyle-coaching class for our patients, entitled Commit to Being Fit. Weight-management courses of this type are delivered nationwide, but we adapted ours specifically for our patients by moving the focus from weight loss to lifestyle choices. To achieve this, our course included visual and tactile educational sessions about things like snack alternatives, portion control, meal planning, and physical activity, whilst taking into consideration our patients' physical abilities, attention deficits, behavioral challenges, and information-accessibility needs.

We held a launch day to introduce the new program and invited patients to sign up. For us, voluntary participation was a key feature of the program and those who came forward each received personalised journals, designed to encourage participation and momentum outside the sessions. Realistic targets were set in a section called My Healthy Food Chart. The program initially consisted of weekly hour-long cooking

sessions, although we soon extended those to two hours to cater better for the patients' diverse range of physical abilities. During the sessions, patients cooked meals together, then ate and reviewed them, so that eating was a fun and social experience. As well as learning new cooking skills, patients developed their fine motor skills, documenting their experiences in their journals.

Loss of the senses of smell and taste are common after brain injury and of course, they're connected – with loss of one sense impacting the other. Early in the program, we offered a tasting session and saw how patients often lacked interest in, and enjoyment of, food. They'd often make the same bland choices from the menu or choose to snack instead, resulting in a diet that didn't provide a balanced variety of nutrients. In response to this, we refocused the sessions to make meals more interesting and imaginative through different colors and textures. We introduced a 'taste-the-rainbow' experience, where patients tried and rated a variety of fruit and vegetables. Those they liked were then included in their personalised meal and snack plan. We also provided group meals that allowed patients to experiment with different herbs, spices, and temperatures, as well as additional flavorings, such as strong cheeses and sauces.

So, what about the outcomes of the program? One obvious measure was the patients' weight. In all cases, this either decreased or remained stable over the twelve weeks. But we also gathered weekly feedback, using evaluation forms after each session. This was generally positive, with patients saying they wished the sessions could be longer and more frequent. A common theme running through their responses was an appreciation of being in a supportive group of their peers, where they felt valued, respected, and had a sense of belonging. For a more objective measure, we also used the Warwick-Edinburgh Mental Wellbeing Scale, which measures mental well-being over time through a seven-part questionnaire. Patients completed this when the program began, and again at six and twelve weeks. All patient scores either remained the same or improved. We reviewed and used these scores, along with soft feedback, to develop future programs.

PAUSE: 10 SECONDS

That is the end of Part C.

That is the end of the Listening test.

OET Mini Listening Test 1 | Simple answer key

Part A

- 1** bus driver
- 2** stretching (sessions)
- 3** glute
- 4** stiffness
- 5** buttock
- 6** heel
- 7** sciatica
- 8** shockwave therapy
- 9** acupuncture
- 10** fibromyalgia
- 11** osteoarthritis
- 12** (the/her) jaw

Part B

- 13** C
- 14** C
- 15** B
- 16** A
- 17** B
- 18** C

Part C

- 19** A
- 20** B
- 21** B
- 22** C
- 23** A
- 24** B

OET Mini Listening Test 1 | Extended answer key

Part A

1: bus driver

'It all started when I got a job as a bus driver.'

2: stretching (sessions)

'So, I started going to the gym... I'd do stretching sessions along with lots of weightlifting'

3: glute

'It's mainly just above the glute and always on that left side'

4: stiffness

'... but my back just doesn't feel right – I guess you'd say there's some kind of stiffness there.'

5: buttock

'It spreads up and down, so that I feel it in my buttock...'

6: heel

'Once or twice, I've woken up with the pain going all the way down my leg – and on one occasion as far as the heel'

7: sciatica

'... at first my doctor said that it was probably sciatica'

8: shockwave therapy

'I started shockwave therapy, and that made an immediate difference'

9: acupuncture

'The other thing she suggested was acupuncture. I'm not sure about that – so I haven't followed it up.'

10: fibromyalgia

'My sister has developed fibromyalgia and I was concerned I might have it too.'

11: osteoarthritis

'he did send me for a blood test and an X-ray. Anyway, to cut a long story short, at that point I was diagnosed with osteoarthritis.'

12: (the/her) jaw

'my mother died of cancer earlier this year. It was in her jaw'

Part B

Question 13: C

M: I've just started taking doxycycline for my acne - I have that with my evening meal.

F: OK – what your describing sounds like heartburn - doxycycline can sometimes cause that if you lie down right after taking it, so I'd recommend having it with breakfast instead

M: Sure.

Option A is wrong because he will still take doxycycline.

Option B is wrong because he agrees to take his medication with breakfast rather than dinner.

Question 14: C

F: Did you report the incident?

M: Yes, I reported it to the lead nurse right away.

F: Good. We'll need to fill out an incident report as well. Can you tell me what steps you took after you realised the error?

Option A is wrong because the problem has already been dealt with.

Option B is wrong because the nurse explains what happened but isn't asked why it happened.

Question 15: B

'She also said it was actually up to me how long I use them for – that I need to see how I get on. I was a bit taken aback by that to be honest. But I do get the point that, ideally, I need to learn to manage without them – so I'll do my best.'

Option A is wrong because although the patient will try to learn to manage without them, he knows that he needs them initially.

Option C is wrong because the patient knows that his son has been briefed about this and says: *'so that shouldn't be a problem.'*

Question 16: A

'I need to get some idea of where this is heading – do I need to make permanent arrangements for her – or is there any chance that he might get over this?'

Option B is wrong because although his grandfather's condition has changed rapidly, he isn't asking why this has happened.

Option C is wrong because although he talks about care for his grandmother, he doesn't ask about his grandfather's discharge.

Question 17: B

'So, we're going to keep him on the ambulance stretcher while we go through the standard IMIST handover, then we're going to move him onto our stretcher. At that point you can each begin your individual assessments'

Option A is wrong because from what he says, we understand that team members clearly already know what their individual duties are.

Option C is wrong because the doctor will be obtaining this information himself.

Question 18: C

F: Have you talked to the pulmonologist?

M: I have. She suggested sticking to the current plan for another day or so, and keeping a close eye on him. If there's no improvement, we may then need to consider additional interventions.

F: Sure.

Option A is wrong because the pulmonologist has already been asked for her view.

Option B is wrong because although this is an option, they don't decide to initiate it today.

Part C

Question 19: A

'... we identified that an often-overlooked area of that recovery was how a person feels about themselves as they adapt to a new lifestyle with the physical, emotional and cognitive scarring caused by their injury'

Option B is wrong because although Nihal talks about a lack of public understanding about brain injuries, he doesn't relate this to the patients' personal relationships.

Option C is wrong because although Nihal explains that patients often '*face a long struggle to regain physical, cognitive and social skills*', he doesn't say that this aspect of their recovery gets too little attention.

Question 20: B

'We realised, however, that disability is the main contributing factor to increased weight gain amongst our residents, and this, in turn, often leads to a diagnosis of type-2 diabetes. For this reason, we decided to launch a weight-management programme.'

Option A is wrong because although Nihal says that health outcomes would be improved by the weight-management programme, he doesn't mention other therapies specifically.

Option C is wrong because Nihal doesn't mention whether or not patients concerns affected the decision to focus on weight management.

Question 21: B

'Weight-management courses of this type are delivered nationwide, but we adapted ours specifically for our patients by moving the focus from weight loss to lifestyle choices.'

Option A is wrong because although Nihal talks about the training the team received, he doesn't say this differed from what happens elsewhere.

Option C is wrong because although Nihal talks about funding and the resources used in the sessions, he doesn't compare these aspects with other programmes.

Question 22: C

'The programme initially consisted of weekly hour-long cooking sessions, although we soon extended those to two hours to cater better for the patients' diverse range of physical abilities'

Option A is wrong because although Nihal talks about how patients volunteered for the programme, this aspect didn't change after the launch day.

Option B is wrong because although Nihal talks about the personalised journals kept by patients, this aspect didn't change after the launch day.

Question 23: A

'we refocussed the sessions to make meals more interesting and imaginative through different colours and textures.'

Option B is wrong because Nihal talks about patients trying different herbs, spices, etc., he doesn't talk about how well these were tolerated.

Option C is wrong because although we understand that the patients had different levels of sensory loss, there is no discussion of how this was assessed.

Question 24: B

'A common theme running through their responses was an appreciation of being in a supportive group of their peers, where they felt valued, respected and had a sense of belonging.'

Option A is wrong because although Nihal talks about measurable targets and objective measures, this wasn't a feature of the weekly patient feedback.

Option C is wrong because although Nihal says patients wished the sessions were longer and more frequent, he doesn't talk about their impact on other forms of therapy.